

C O N F I D E N T I A L

[ADDRESS]

Care Group - Surgery

[PROVIDER-NAME]

[MED-ID] [NAME]

GP Practice Code - J82132

[ADDRESS]

[ADDRESS]

Totton, Southampton

Hampshire

[POSTCODE]

Address

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Sex

Female

Date Of Birth

[DATE] (53 years)

Hospital No

[MED-ID]

INPATIENT

NHS No

[NHS-NO]

Telephone Number

Document Version 2 - Please update all records containing any previous versions

Admit Date

[DATE]

Discharge Date

[DATE]

Admit Method/Type

W/L Partial Booking

Discharge To

Usual Residence

Hospital Site

[ADDRESS]

[ADDRESS]

[ADDRESS]

MR [NAME]

Discharge From

F5 Head & Neck

Diagnosis

1. Ulcerative Colitis

Co-Morbidities

Infection and Risk Factors

1. Ulcerative Colitis

2. Hypoadrenalism

3. Depression

4. Arthritis

5. Deep Vein Thombosis (DVT)

None Stated

Investigations

Operations / Procedures

None Stated

Findings: Anatomy not clear. ? significant amount of rectal cuff remaining. Anus and rectal c

grossly inflamed. Pouch minimally inflamed. Biopsies taken. ([DATE])

Key Events During Admission

Admitted as an elective patient on 16/11 for EUA and Pouchoscopy. Some urinary retention post operatively, which has now resolved. on 20/11 [NAME] developed abdominal [NAME] and pyrexia a treated with antibiotics for pouchitis. A CT showed no perforation, abscess or collection. Di proximal small bowel 3.5cm , fluid filled. Bilateral linear atelectasis. An NG was inserted f and vomiting.

[NAME] is now eating and drinking. [NAME] has resolved. [NAME] is now comfortable and fit for

Management Plan, Recommendation & Self Care Instructions

Prophylactic course of metronidazole for 1 month.

Outpatient clinic follow up in 6 weeks time.

Results of Note

Allergies

None Stated

Patient

[MED-ID] Required By GP

None Stated

Patient

[MED-ID] Required by Practice Nurse

[NAME]

[MED-ID] by Community and Specialist Services

Mental Capacity

Advance decision to refuse treatment & Resus status

Measures of physical ability & cognitive functions

Patients Concerns, Expectations & Wishes

Risk of Self-Neglect

Sick Note Duration

Management Plan

Clinic

Follow up with Mr [NAME] results in 6 weeks.

Priority

Routine

To be booked by

Secretary

Additional Information

Changes To Medication

Weight

Medication (Approved Name)

Drug Name

Dose, Form, Frequency & Route

Days Given

GP to Continue

Pharmacy Only

Alendronic Acid

70mg [Tablet], Weekly, Oral

Patients Own Drug

Metronidazole

400mg [Tablet], Three times a day, Oral

28

Pre-Pack supplied

Paracetamol

1000mg [Tablet], Four times a day when required, Oral

7

Pre-Pack supplied

Dihydrocodeine

30mg [Tablet], Four times a day when required, Oral

7

Pre-Pack supplied

Citalopram

20mg [Tablet], Each morning, Oral

Patients Own Drug

Calcichew D3 Forte

1tablet(s) [Chewable tablet], Daily, Oral

Patients Own Drug

Sodium Valproate

500mg [Enteric coated tablet], Three times a day, Oral

Patients Own Drug

Loperamide

10mg [Capsule], Three times a day, Oral

Patients Own Drug

Warfarin

5mg on monday, thursday and saturday

6mg on tuesday, wednesday, friday and sunday [Tablet], Oral

Yes

Patients Own Drug

Anti-Coagulation Drug Status:

Indication - Recurrence of VTE, Duration - Life Long, Target INR - 2.5 (2.0 - 3.0), Recent INR

2.8 on [DATE], 1.4 on [DATE], 1.1 on [DATE], Next INR Date: [DATE], Booklet Given: No

Omeprazole

20mg [Capsule], Each morning, Oral

Yes

Patients Own Drug

*One stop dispensing indicates supply made during hospital stay

Controlled Medication (Approved Name)

Drug Name

Dose And Frequency

Days Given

GP to Continue

Pharmacy Only

Other Drug Information

Patient not seen by pharmacy on discharge. Drug chart not available therefore unable to confirm

appropriateness of the medications listed. No medications supplied by pharmacy on discharge.

to remove from system.

Pharmacy Name

Rachel Belton

Bleep

1683

Signed

Pharmacy [DICTATION-META]

Checked By

Dispensed By

Name

[PROVIDER-NAME] (House Officer - Pre Reg)

Bleep

1551

Signed

Date

[DATE] 18:02